

Paediatric population

The effects of long-term (greater than 36 weeks) gabapentin therapy on learning, intelligence, and development in children and adolescents have not been adequately studied. The benefits of prolonged therapy must therefore be weighed against the potential risks of such therapy.

Abuse and dependence

Cases of abuse and dependence have been reported. Carefully evaluate patients for a history of drug abuse and observe them for possible signs of gabapentin abuse e.g. drug-seeking behaviour, dose escalation, development of tolerance.

Laboratory tests

False positive readings may be obtained in the semi-quantitative determination of total urine protein by dipstick tests. It is therefore recommended to verify such a positive dipstick test result by methods based on a different analytical principle such as the Biuret method, turbidimetric or dye-binding methods, or to use these alternative methods from the beginning.

Effects on the ability to drive and use machines

Gabapentin may impair the ability to drive a car or operate potentially dangerous machinery. Patients are advised not to drive or operate potentially dangerous machinery, until it is known that this medication does not affect their ability to engage in these activities.

Interactions with other medicaments

There are reports of respiratory depression and/or sedation associated with gabapentin and opioid use. In some of these reports, the authors considered this a particular concern with the combination of gabapentin and opioids, especially in elderly patients.

Patients who require concomitant treatment with opioids should be carefully observed for signs of CNS depression, such as somnolence, sedation and respiratory depression and the dose of gabapentin or opioid should be reduced appropriately.

No interaction between gabapentin and phenobarbital, phenytoin, valproic acid, or carbamazepine has been observed.

Co-administration of gabapentin with oral contraceptives containing norethindrone and/or ethinyl estradiol, does not influence the steady-state pharmacokinetics of either component.

Co-administration of gabapentin with antacids containing aluminium and magnesium, reduces gabapentin bioavailability up to 24%. It is recommended that gabapentin be taken at the earliest two hours following antacid administration.

Renal excretion of gabapentin is unaltered by probenecid.

A slight decrease in renal excretion of gabapentin that is observed when it is co-administered with cimetidine is not expected to be of clinical importance.

Pregnancy and lactation

Pregnancy:

There is insufficient clinical experience with gabapentin in pregnancy to confirm its safety in this patient population. Since gabapentin is frequently prescribed with other anticonvulsants, a clear association between maternal gabapentin use and fetal adverse effects cannot be determined. Due to lack of adequate, well-controlled studies, hence it is recommended that gabapentin should be used during pregnancy only if the potential benefit outweighs the potential risk to the fetus.

Lactation:

Gabapentin is excreted in human milk. Because the effect on the nursing infant is unknown, caution should be exercised when gabapentin is administered to a nursing mother. Gabapentin should be used in nursing mothers only if the benefits clearly outweigh the risks.

Side effects

The adverse reactions observed (adjunctive and monotherapy) and neuropathic pain have been provided in a single list below by class and frequency (very common (≥ 1/10), common (≥1/100 to <1/10), uncommon (≥1/1000 to <1/100), rare (≥1/10,000 to <1/1,000), very rare (<1/10,000), not known (cannot be estimated from the available data).

Additional reactions reported from post-marketing experience are included as frequency Not known (cannot be estimated from the available data) in *italics* in the list below.

Within each frequency grouping, undesirable effects are presented in order of decreasing seriousness.

Infections and infestations

Very Common: Viral infection

Common: Pneumonia, respiratory infection, urinary tract infection, infection, otitis media

Blood and lymphatic system disorders

Common: leucopenia

Not known: *thrombocytopenia*

Immune system disorders

Uncommon: allergic reactions (e.g. urticaria)

Not known: hypersensitivity syndrome (a systemic reaction with a variable presentation that can include fever, rash, hepatitis, lymphadenopathy, eosinophilia, and sometimes other signs and symptoms), anaphylaxis.

Metabolism and nutrition disorders

Common: anorexia, increased appetite

Uncommon: hyperglycaemia (most often observed in patients with diabetes)

Rare: hypoglycaemia (most often observed in patients with diabetes)

Not known: hyponatraemia.

Psychiatric disorders

Common: hostility, confusion and emotional lability, depression, anxiety, nervousness, thinking abnormal

Uncommon: agitation

Not known: hallucinations

Nervous system disorders

Very common: somnolence, dizziness, ataxia,

Common: convulsions, hyperkinesias, dysarthria, amnesia, tremor, insomnia, headache, sensations such as paresthesia, hypaesthesia, coordination abnormal, nystagmus, increased, decreased, or absent reflexes

Uncommon: hypokinesia

Rare: loss of consciousness

Not known: other movement disorders (e.g. choreoathetosis, dyskinesia, dystonia)

Eye disorders

Common: visual disturbances such as amblyopia, diplopia

Ear and labyrinth disorders

Common: vertigo

Not known: *tinnitus*

Cardiac disorders

Uncommon: palpitations

Vascular disorders

Common: hypertension, vasodilatation

Respiratory, thoracic and mediastinal disorders

Common: dyspnoea, bronchitis, pharyngitis, cough, rhinitis

Rare: respiratory depression

Gastrointestinal disorders

Common: vomiting, nausea, dental abnormalities, gingivitis, diarrhoea, abdominal pain, dyspepsia, constipation, dry mouth or throat, flatulence

Not known: pancreatitis

Hepatobiliary disorders

Not known: hepatitis, jaundice

Skin and subcutaneous tissue disorders

Common: facial oedema, purpura most often described as bruises resulting from physical trauma, rash, pruritus, acne

Not known: Stevens-Johnson syndrome, angioedema, erythema multiforme, alopecia, drug rash with eosinophilia and systemic symptoms (see Special warnings and precautions for use)

Musculoskeletal and connective tissue disorders

Common: arthralgia, myalgia, back pain, twitching

Not known: rhabdomyolysis, myoclonus

Renal and urinary disorders

Not known: acute renal failure, incontinence

Reproductive system and breast disorders

Common: impotence

Not known: breast hypertrophy, gynaecomastia, sexual dysfunction (including changes in libido, ejaculation disorders and anorgasmia).

General disorders and administration site conditions

Very common: fatigue, fever

Common: peripheral oedema, abnormal gait, asthenia, pain, malaise, flu syndrome

Uncommon: generalized oedema

Not known: withdrawal reactions (mostly anxiety, insomnia, nausea, pains, sweating), chest pain. Sudden unexplained deaths have been reported where a causal relationship to treatment with gabapentin has not been established.

Investigations

Common: WBC (white blood cell count) decreased, weight gain

Uncommon: elevated liver function tests SGOT (AST), SGPT (ALT) and bilirubin

Not known: blood creatine phosphokinase increased

Injury, poisoning and procedural complications

Common: accidental injury, fracture, abrasion

Uncommon: fall

In patients on haemodialysis due to end-stage renal failure, myopathy with elevated creatine kinase levels has been reported.

Respiratory tract infections, otitis media, convulsions and bronchitis were reported only in children. Additionally, in children, aggressive behaviour and hyperkinesias were reported commonly.

Symptoms and Treatment of over dose

Symptoms

Acute, life-threatening toxicity has not been observed with gabapentin overdoses of up to 49 g. Symptoms of the overdoses included dizziness, double vision, slurred speech, drowsiness, loss of consciousness, lethargy and mild diarrhoea. All patients recovered fully with supportive care. Reduced absorption of gabapentin at higher doses may limit drug absorption at the time of overdosing and, hence, minimize toxicity from overdoses.

Overdoses of gabapentin, particularly in combination with other CNS depressant medications, may result in coma.

Treatment

Treatment of gabapentin exposure is largely supportive in nature with careful attention to airway protection in severe cases.

Although gabapentin can be removed by hemodialysis, based on prior experience it is usually not required. However, in patients with severe renal impairment, hemodialysis may be indicated.

Storage condition

Store below 30°C.

Shelf life

36 months.

Presentation:

Each blister pack contains 10 Capsules. Available in pack sizes of 5 blister of 10's and 10 blister of 10's.



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